

August 12, 2026

Aetna
Provider Resolution Team
P.O. Box 14020
Lexington, KY 40512

RE: Formal Appeal of Claim Denial — Stewart Stanton (MRN MUSC1937610), Claim MUSC-20251127-ECAE-1

Patient	Stewart Stanton (DOB 1915-07-03)
MRN	MUSC1937610
Member ID / Group	AET842953112 / GRP-70070
Claim number	MUSC-20251127-ECAE-1
Date of service	2025-11-27
Procedure billed	CPT 99214 — Office/outpatient visit, established patient, moderate complexity
Primary diagnosis	C80.1 — Malignant (primary) neoplasm, unspecified
Denied amount	\$611.63
Denial code	CARC 50 / RARC N115 — These are non-covered services because this is not deemed a 'medical necessity' by the payer.
Appeal deadline	2026-07-03

Dear Appeals Review Committee:

SUMMARY OF APPEAL

MUSC Health submits this Level 1 reconsideration on behalf of patient Stewart Stanton (MRN: MUSC1937610, Member ID: AET842953112, Group: GRP-70070) regarding claim MUSC-20251127-ECAE-1 for a date of service of November 27, 2025, at MUSC Health Rutledge Tower Ambulatory Care. The claim was denied on January 4, 2026, under CARC 50 ("These are non-covered services because this is not deemed a 'medical necessity' by the payer") with RARC N115, citing a Local Coverage Determination, and accompanied by the payer remark: "Documentation submitted does not establish that the service was reasonable and necessary for the diagnosis reported." MUSC Health respectfully disagrees with this determination and requests full reversal and payment of the billed amount of \$611.63.

CLINICAL BACKGROUND

Mr. Stanton is an established patient of rendering provider Daniel R. Okafor, MD (NPI 1558493021) presenting with a complex, active oncologic and metabolic profile. His primary diagnosis is malignant neoplasm (C80.1), reflecting an active malignancy of the colon with onset documented as far back as September 1966. His oncologic history includes partial resection of the colon, rectal polypectomy, colonoscopy, and a series of combined chemotherapy and radiation therapy treatments. Active medications include oxaliplatin injection and leucovorin injection, both consistent with ongoing oncologic management. In addition to his malignancy, Mr. Stanton carries multiple active comorbidities requiring coordinated management, including diabetes mellitus with neuropathy, diabetic renal disease, chronic kidney disease, metabolic syndrome, and hypertriglyceridemia, and he is maintained on insulin therapy and hydrochlorothiazide. The recurrent rectal polyp (secondary diagnosis R69) represents an additional active condition under surveillance. This constellation of serious, interrelated conditions clearly supports the need for ongoing, moderate-complexity outpatient evaluation and management.

BASIS FOR APPEAL

The service billed — CPT 99214, an established patient office visit of moderate complexity — is entirely consistent with the documented clinical picture. CPT 99214 is appropriate when the medical decision-making involves a problem of moderate complexity, which is readily supported by an active primary malignancy managed with chemotherapy agents, concurrent diabetic renal disease, chronic kidney disease, neuropathy, and metabolic syndrome. The selection of this level of service reflects the breadth and severity of conditions that must be assessed, coordinated, and managed at each encounter.

Aetna's denial asserts that the documentation does not establish the service as reasonable and necessary for the diagnosis reported. However, an outpatient evaluation and management visit for an established patient with an active malignancy and multiple serious comorbidities is, by any generally accepted clinical standard, both reasonable and necessary. Ongoing physician oversight is essential to monitor treatment response, manage medication-related toxicities, assess renal function in the context of nephrotoxic chemotherapy agents, and coordinate care across the patient's active conditions. The denial does not identify any specific documentation deficiency, nor does it articulate a clinical rationale for concluding that physician evaluation was unnecessary. A blanket medical necessity denial unsupported by specific clinical reasoning does not meet the standard of a thorough, individualized coverage determination.

REQUESTED ACTION

MUSC Health respectfully requests that Aetna conduct a complete clinical review of the submitted documentation and reverse the denial in full. We ask that claim MUSC-20251127-ECAE-1 be reprocessed and payment issued at the allowed amount of \$425.70, consistent with the Aetna Choice Plus contracted rate, with any applicable remittance issued promptly in accordance with applicable prompt-pay obligations. Should Aetna require additional clinical documentation to complete this review, we ask that a specific and itemized request be directed to our office at the earliest opportunity so that we may respond within the appeal timeframe of July 3, 2026.

Respectfully submitted,

Angela R. Whitfield, RN, BSN, CPC

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MUSC Health — Revenue Cycle / Patient Financial Services
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Enclosures

- Remittance advice / explanation of payment
- Itemized claim detail (UB-04 / CMS-1500)
- Relevant clinical documentation from the medical record